



MFMM

41

Support

OBSTETRICS
OTHER Q
SUPPORT 41

Inverted Questions

Family Medicine

- Select suitable contraception method:

- Case 1

A 40-year-old woman (para 3+0) comes to your office for her 6-week postpartum visit. She had an uncomplicated pregnancy, normal spontaneous vaginal delivery, and routine postpartum course. She and her baby are doing well. She has not gotten her period yet. She is exclusively breastfeeding. She does not want to get pregnant again. The patient has no major medical problems, does not smoke, and has already returned to her aerobics class. She has no history of STDs or abnormal Pap tests. She desires a reliable birth control method that she does not have to remember to take every day or remember to use every time she has sex with her husband. Her examination is completely normal.

- Case 2

A 34-year-old woman (P2+0) who is a long-term patient of yours presents to the office for a routine blood pressure check. She was recently diagnosed with hypertension and diabetes. Her medications include metformin, hydrochlorothiazide, and a multivitamin. Her blood pressure today is 150/100 mm Hg, and her body mass index is 30. She uses the “calendar” method only. She reports that her menses have been irregular and vary from 20 to 45 days apart. The patient is worried about the risks of hormonal contraception given her medical conditions. She does not want to be pregnant for several years.

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- Case 3

A 21-year-old woman (gravida 0, para 0) walks into your office on a Tuesday morning. She tells you that she had sexual intercourse Friday night. They used a condom, but it broke. Her last menstrual period was approximately 3 weeks ago and was normal in flow and duration. She would be devastated if she got pregnant. She is a heavy smoker (two packs per day) but otherwise has no medical problems, denies bleeding or other symptoms, and her examination is normal. Her most recent Pap smear and gonococcus/chlamydia results were normal.

- Case 4

27 years old female (P1+0) is using COCs (combined oral contraceptives) for 1 year. She has one child, now 2 years old. She presents to you after missing some pills because of social troubles.

How can you help her with her missed pills?

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Which of the following is NOT a component of premarital care?

- a) Genetic counselling
- b) Inter-conception care
- c) Basic lab investigations
- d) Promotion for proper ANC
- e) Control and management of chronic diseases

Which of the following is NOT included in the counseling sessions provided for the couples during premarital care?

- a) Genetic counselling
- b) Relevant health problem counselling
- c) Contraception counselling
- d) Healthy lifestyle change counselling

Which of the following methods is the correct way to calculate the estimated date of delivery (EDD)?

- a) First day of last menstrual period (LMP) + 8 months + 1 week
- b) First day of last menstrual period (LMP) + 9 months
- c) Last day of last menstrual period (LMP) + 8 months + 1 week
- d) First day of last menstrual period (LMP) + 8 months
- e) First day of last menstrual period (LMP) + 9 months + 1 week

A 38-year-old patient asks if she has to "hurry up" and get pregnant early after marriage. Which of the following is TRUE regarding her age?

- a) Advanced maternal age is not a contributor to infertility
- b) Preeclampsia and toxemia affect very young women and spare older women
- c) Diabetes mellitus is more common in advanced maternal age
- d) The incidence of fetal anomalies increases with age, whereas the number of abortions decreases
- e) As long as the patient is healthy, postponing pregnancy till age 40 is possible

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During ANC for a 27-year-old primigravida with BMI 20, she is concerned about weight gain during pregnancy. What is the recommended weight gain?

- a) 7 to 11.5 kg
- b) 12.5 to 18 kg
- c) 11.5 to 16 kg
- d) 5 to 9 kg
- e) None of the above

At what stage of gestation would you expect a multiparous woman to begin to feel fetal movements?

- a) 12–14 weeks
- b) 16–18 weeks
- c) 18–20 weeks
- d) 22–24 weeks

You are caring for a 33-year-old woman, 12 weeks pregnant, who smokes 2–3 cigarettes/day and drinks about 3 cups of alcohol per week. Which of the following is TRUE?

- a) Limit alcohol intake to one or two glasses of wine is advisable
- b) Children with fetal alcohol syndrome have developmental, behavioral, and learning problems
- c) Alcohol consumption in the third trimester is not directly associated with fetal alcohol syndrome
- d) Smoking must be discontinued at least 3 months before pregnancy to avoid its hazards

A 22-year-old primigravida with no psychiatric history delivers a healthy baby at 41 weeks and develops insomnia, anxiety, and crying spells on postpartum day 4. What is the most likely diagnosis?

- a) Postpartum blues
- b) Postpartum depression (PPD)
- c) Postpartum psychosis
- d) Alcohol withdrawal

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All of the following are risk factors for PPD EXCEPT:

- a) Prior psychiatric disease of any kind
- b) The postpartum blues
- c) Cesarean section
- d) Exclusive bottle feeding
- e) Infant with special needs

A pregnant lady came for her ANC visit at 24 weeks of gestation. What is the timing for her next visit according to the schedule of the minimum eight ANC visits?

- a) 26–30 weeks
- b) At least three weeks after the first visit
- c) 30–34 weeks
- d) At 36 weeks
- e) The next visit is scheduled according to patient availability

A 29-year-old healthy, nulliparous woman stopped using OCPs recently and now depends inconsistently on condoms and withdrawal. She smokes one pack/day and her BMI is 31. Which of the following statements is NOT true regarding the use of combined hormonal contraceptives for this patient?

- a) It is associated with a significantly decreased risk of PID
- b) History of chlamydia is not a contraindication for hormonal contraception
- c) Decreases her risk of endometrial cancer
- d) It is contraindicated because of her obesity
- e) Estrogen may improve her acne

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Your patient wants a reliable birth control method that does not require daily use or use during every sexual encounter. Her examination is completely normal. Which would be the most appropriate contraceptive method for this patient at this time?

- a) Bilateral tubal ligation (BTL) or vasectomy
- b) Vaginal contraceptive ring
- c) Progesterone-only pills
- d) A levonorgestrel IUD or Depo-Provera

You started a 20-year-old woman on COCs 2 months ago. She returns asking to discontinue them due to side effects. Which of the following is the most frequent reason for discontinuing COCs?

- a) Irregular bleeding
- b) Breast tenderness
- c) Fluid retention
- d) Headache
- e) Nausea

Which of the following statements would NOT be regarded as reasonable nutritional advice for a pregnant woman?

- a) Supplementation with calcium (1000–1300 mg per day)
- b) Supplementation with iron, especially if anemia is detected
- c) Supplementation with folic acid 0.4 mg daily throughout pregnancy
- d) Supplementation with vitamin A
- e) Supplementation with vitamin D if sunlight exposure is limited

A 42-year-old pregnant woman is now 17 weeks and missed her combined test for Down's syndrome screening. You counsel her about the quadruple test. Which of the following is/are TRUE about the quad test?

- a) The best time for the quad test is after 20 weeks of gestation
- b) Unconjugated oestradiol, ACHT, AFP, and inhibin A
- c) Beta hCG, AFP, nuchal translucency, and inhibin A
- d) AFP, inhibin B, beta hCG, and oestradiol
- e) Can be done from 15–20 weeks of gestation

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- A 37-year-old pregnant woman came for her first ANC visit. She had been trying to get pregnant for 9 years and has a history of PCOS and irregular menstruation. Her last menstrual period was on 6th January and she has a positive home pregnancy test.

Which of the following statements are TRUE or FALSE?

1. Her estimated due date (EDD) will be 13th October.
 2. This lady is considered as having a high-risk pregnancy.
 3. Ultrasound is mandatory in the 18 ANC visit for this lady.
- What services can you offer a 25-year-old woman planning for marriage in 4 months?
 - What are three important items in history taking during premarital screening?
 - What are the routine investigations done in premarital screening?
 - What are the counseling sessions provided for couples during premarital care?
 - What is the importance of antenatal care (ANC)?
 - What are the components of antenatal care?
 - What are the proper timings for ANC visits?
 - What is the value of ultrasound testing during each trimester of pregnancy?
 - What are two sure signs of pregnancy?

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- What are the important physical examinations during repeated ANC visits?
- What are the important educational messages for the pregnant female?

Which of the following is not a component of premarital care?

- a) Genetic counselling
- b) Promotion for proper ANC
- c) Basic lab investigations
- d) Screening for substance abuse
- e) Control risk factors for high-risk pregnancy

When should inter-conception care be provided?

- a) Six months after the previous pregnancy
- b) During routine postpartum care
- c) Inter-conception care visits should be scheduled with the female during ANC visits
- d) During the child immunization visits

Which of the following methods is the correct way to calculate the estimated date of delivery (EDD)?

- a) First day of last menstrual period (LMP) + 3 months + 1 week
- b) First day of last menstrual period (LMP) + 9 months
- c) Last day of last menstrual period (LMP) + 8 months + 1 week
- d) First day of last menstrual period (LMP) + 8 months
- e) First day of last menstrual period (LMP) + 9 months + 1 week

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The primary purpose of prenatal care is to ensure a better outcome; a healthy baby and minimal risk to the mother. Which of the following is not important to achieve this objective?

- a) Early, accurate assessment of gestational age
- b) Regular visits focusing on health and well-being of the mother
- c) Identification of patients at risk (e.g., gestational diabetes, hypertension)
- d) Performance of anatomic ultrasound in the second trimester
- e) Attention to emotional health of the patient and family

During the physical examination for a pregnant woman during ANC, which of the following findings increases the risk of the pregnancy?

- a) Elevated blood pressure
- b) A retroverted uterus
- c) A thrombosed hemorrhoid
- d) Obesity
- e) Detected murmur of mitral stenosis

At what stage of gestation would you expect a multiparous woman to begin to feel fetal movements?

- a) At 12–14 weeks
- b) At 16–18 weeks
- c) At 18–20 weeks
- d) At 20–22 weeks
- e) After 22 weeks

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A 38-year-old woman asks if she should hurry up and get pregnant after marriage. Which of the following is true regarding her age?

- a) Advanced maternal age is not a contributor to infertility
- b) Preeclampsia and toxemia affect very young, usually nulliparous women and spare older women
- c) Diabetes mellitus is more commonly found in advanced maternal age
- d) The incidence of fetal anomalies increases with age, whereas the number of abortions decreases
- e) As long as your patient is in good health, exercises regularly, and does not have a chronic disease, postponing pregnancy till age 40 is possible

Which of the following statements regarding alcohol consumption in pregnancy and fetal alcohol syndrome is/are true?

- a) Limit alcohol intake to one or two glasses of wine
- b) Abstain completely from alcohol during pregnancy
- c) Children with fetal alcohol syndrome have developmental and behavioral problems
- d) Alcohol consumption in the third trimester is not directly associated with fetal alcohol syndrome
- e) Children with fetal alcohol syndrome have major facial features

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- A 37 years old pregnant lady came for her 1st antenatal care visit, she has been trying to get pregnant for 9 years. She suffered from PCOs and irregular menstruation. Her last menses was in 6th January. She has a +ve home pregnancy test.

True or False:

1. Her EDD will be 13th October 2022.

2. This lady is considered as having a high risk pregnancy.

3. U/S is mandatory in the 1st ANC visit for this lady.

- What is the value of US examination?
- What Ultrasound dating is considered accurate to do within?

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What is the value of Hb % which is considered normal for first trimester pregnant female?

- a) 10 gm/dl
- b) 12 gm/dl
- c) 13 gm/dl

A 24-year-old woman presents for an initial prenatal visit. She is at 9 weeks' gestation based on her LMP. She had a previous pregnancy affected by a neural tube defect. What is the most likely prescription regarding folic acid?

- a) 4 g of folic acid once per day to be started at the first trimester of pregnancy
- b) 5 mg of folic acid once per day beginning at least one month prior to conception and continuing through the first trimester
- c) 0.6 mg folic acid once per day beginning at least one month prior to conception and continuing through pregnancy

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- A 28-year-old pregnant female (G2 P1+0) comes for her initial ANC visit at 8 weeks' gestation with no specific complaints. Her pre-pregnancy weight was 62 kg. On examination:

Temp: 37°C

HR: 80

RR: 17

BP: 110/70

Weight: 63 kg

Height: 162 cm

Hb: 12 g/dl

Answer the following:

What is the expected normal weight gain at the end of pregnancy?

What are the components of healthy eating during pregnancy you want to tell her about?

What are the nutritional supplements you will prescribe for her during the pregnancy?

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- A 25-year-old pregnant female (G2 P1+0) presents for her second prenatal visit at 16 weeks' gestation. She has no specific complaint. On examination:

Pallor of lips and palm

Temp: 37.8°C

HR: 95

RR: 18

BP: 110/70

Hb: 9 g/dl

MCV: 70 μ m

MCH: 23 pg

Answer the following:

What is the treatment of the most likely diagnosis of this case?

What is the indication for giving IV iron for this pregnant female?

What is the expected response after receiving the treatment?

How to follow up the female with this medical problem?

What are the causes of failure to reach the expected response during the therapy?

What are the indications for referral regarding this medical problem?

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You are caring for a 15-year-old primiparous patient at 8 weeks' gestation. The patient is 164 cm tall and weighs 50 kg. The patient asks why she is supposed to gain so much weight. What is the best response?

- a) Gaining 8–12 kg is recommended for healthy fetal growth.
- b) Inadequate weight gain delays lactation after delivery.
- c) Weight gain is important to assure that you get enough vitamins.
- d) Inadequate weight gain can lead to decreased fetal growth and development.

You are working with a pregnant teen. In order to accurately assess her nutritional intake, you should:

- a) Assess laboratory values
- b) Ask her to complete a 24-hour dietary recall
- c) Observe for clinical signs of malnutrition
- d) Ask about her cooking facilities

Women with inadequate weight gain during pregnancy are at higher risk of giving birth to an infant with:

- a) Spina bifida
- b) Intrauterine growth restriction
- c) Diabetes mellitus
- d) Down syndrome

The purpose of a nutritional assessment does not include which of the following?

- a) Identify individuals who are malnourished
- b) Provide data for designing a nutrition plan of care that will prevent or minimize the development of malnutrition
- c) Identify those who are at risk for malnutrition
- d) Establish a baseline data for evaluating the efficacy of nutritional care
- e) None of the above

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Which is not true regarding the comprehensive nutritional assessment?

- a) Is performed for all patients
- b) Includes dietary history and clinical information
- c) Includes physical examination
- d) Includes anthropometric measures
- e) Includes laboratory tests

Which of the following is the easiest and most popular method for obtaining information about dietary intake?

- a) Talking to your patient
- b) Food frequency questionnaire
- c) 3-day food diary
- d) 24-hour recall
- e) Direct observation

Which of the following is a common error that occurs with the 24-hour diet recall?

- a) The individual or family member may not be able to recall the type or amount of food eaten
- b) The truth may be altered for various reasons
- c) The intake within the last 24 hours may be atypical of usual intake
- d) All of the above

Anthropometry is:

- a) Absent menstrual cycles
- b) A measurement and evaluation of growth, development, and body composition
- c) A serious side effect of steroids which affects, among other areas, the skin, eyes, and gums
- d) Scanty menstrual flow
- e) An abnormally large head circumference, which often signifies disease

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Recommended weight gain based on pre-pregnant BMI:

Underweight (BMI < 19.5)

Normal (BMI 19.5–24.9)

Overweight (BMI 25–29.9)

A pregnant woman with a body mass index (BMI) of 22 asks how she should be gaining weight during pregnancy. The BEST response would be to tell the woman that her pattern of weight gain should be approximately:

- a) Gaining 11.5–16 kg is recommended for healthy fetal growth
- b) A kilogram a week throughout pregnancy
- c) Adequate weight gain is 3 kg per week during the first two trimesters, then 0.4 kg per week during the third trimester
- d) Recommended gain at the rate of approximately 0.4 kg per week in the first and second trimesters
- e) Gaining 12.5–18 kg is recommended for healthy fetal growth

A 22-year-old woman pregnant with a single fetus has a preconception body mass index (BMI) of 24. When she was seen in the clinic at 14 weeks of gestation, she had gained 3.8 kg since conception. How would the nurse interpret this?

- a) This weight gain indicates possible gestational hypertension
- b) This weight gain indicates that the woman's infant is at risk for intrauterine growth restriction (IUGR)
- c) This weight gain cannot be evaluated until the woman has been observed for several more weeks
- d) The woman's weight gain is appropriate for this stage of pregnancy

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You are preparing an antenatal nutrition class for pregnant women. Which material should be included in the teaching?

- a) Dietary protein can only be obtained through consuming dairy, meat, and eggs
- b) During pregnancy, consumption of oily fish should be avoided
- c) Iron absorption is generally higher for vegetable products than for animal products
- d) Nutritional iodine requirements generally can be met through intake of iodized salt

Which statement is best to include when teaching a pregnant adolescent about nutritional needs of pregnancy?

- a) It is important to eat iron-rich foods like meat every day.
- b) Calcium and milk aren't needed until the third trimester.
- c) Folic acid intake is the key to having a healthy baby.
- d) You just need to pay attention to what you eat now.

Which minerals and vitamins usually are recommended to supplement a pregnant woman's diet?

- a) Fat-soluble vitamins A and D
- b) Water-soluble vitamins C and B6
- c) Iron and folate
- d) Calcium and zinc

When planning a diet with a pregnant woman, the FIRST action would be to:

- a) Review the woman's current dietary intake.
- b) Teach the woman about the food pyramid.
- c) Caution the woman to avoid large doses of vitamins, especially those that are fat-soluble.
- d) Instruct the woman to limit the intake of fatty foods

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What is a food pyramid?

- a) It is a representation of the optimal number of servings that a person should eat each day from each of the basic food groups.
- b) It is a representation of different types of food groups available in our ecosystem
- c) It is a broad term that shows the hierarchy of food and nutrition levels
- d) It is a chart that shows available nutrients in various vegetables.

Which food group should you eat the most of?

- a) Dairy
- b) Vegetables
- c) Meats
- d) Grain
- e) Fats, Oils, and Sweets

When counseling a client about getting enough iron in her diet, you should tell her that:

- a) Milk, coffee, and tea aid iron absorption if consumed at the same time as iron.
- b) Iron absorption is inhibited by a diet rich in vitamin C.
- c) Iron supplements are permissible for children in small doses.
- d) Constipation is common with iron supplements

Several of the pregnant patients have been diagnosed with iron-deficiency anemia. Which should you encourage the pregnant to consume more of in order to increase iron absorption?

- a) Hot soup
- b) Orange juice
- c) Milk
- d) Green tea

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In case of providing nutritional counseling for a pregnant woman with a hemoglobin of 8.0. Which statement indicates that additional teaching is necessary?

- a) My iron is low, but it will increase as I take iron supplements.
- b) I need to increase food sources that contain iron.
- c) If I drink lots of milk, I will increase my iron level faster.
- d) I might feel less energetic and tire more easily while my iron is low.

Iron deficiency anemia is associated with higher preterm births. What can a mother do to receive adequate amounts? (Select all that apply)

- a) Take iron supplements with orange juice
- b) Intake 20 mg/day
- c) Intake lean meats, eggs
- d) Intake dark green leafy vegetables

Obstetrics

Which of the following suggests worsening cardiac status in a pregnant patient?

- a) Mild shortness of breath
- b) Dependent edema
- c) Increased pulse rate
- d) Rales on lung examination
- e) Malar flush of the face

Termination of pregnancy is advisable in a woman having which of the following heart diseases?

- a) Atrial septal defect
- b) Aortic stenosis
- c) Aortic regurgitation
- d) Eisenmenger's syndrome
- e) Mitral stenosis

A baby is born to a mother having structural heart defect. What will be the chances of developing structural heart defect in the baby?

- a) < 1%
- b) 1%
- c) 2–4%
- d) 5–10%
- e) 10%

Which of the following is the investigation of choice during pregnancy for detection of structural cardiac abnormalities?

- a) Doppler studies
- b) Chest X-ray
- c) ECG
- d) Echocardiography
- e) Thallium scan

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A primigravida presents in the pre-pregnancy counseling clinic. She had a prosthetic valve replacement surgery three years ago.

Which one of the following statements is correct about her antenatal care?

- a) She will have to take warfarin throughout pregnancy
- b) Breastfeeding will be contraindicated if she takes warfarin
- c) Maternal mortality is around 50%
- d) Mode of delivery will be cesarean section
- e) Prophylactic antibiotics will be administered to prevent endocarditis

A primigravida having mitral stenosis is in active labor.

Which one of the following should not be done during her management?

- a) Mandatory application of forceps or ventouse
- b) Strict record of fluid intake and output
- c) Continuous electronic fetal heart rate monitoring
- d) Active management of third stage with oxytocin
- e) Prophylactic antibiotics

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1. List four major causes of Cesarean section
2. List two contraindications of CS
3. List three types of anesthesia for CS
4. List two types of cesarean sections either according to uterine incision or timing of the procedure
5. List three uterine incisions which can be done during CS
6. Name the instrument referred to by blue arrow which was used during shown CS image. Why is it used?



7. List the range of uterine rupture in deliveries following upper segment cesarean section
8. List the range of uterine rupture in deliveries following lower segment cesarean section
9. At what gestational age is elective CS recommended to be done?
10. List 5 prerequisites before forceps applications
11. List four complications of forceps application

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Which is the incorrect statement regarding instrumental vaginal delivery?

- a) More common with the usage of epidural anesthesia
- b) Vacuum extraction requires less pain relieving than forceps delivery
- c) Proper usage of oxytocin limits the need for instrumental delivery
- d) Increases the rate of perineal tears
- e) Maternal trauma is more frequent with vacuum extractor than with obstetric forceps

Which is a contraindication for forceps delivery?

- a) Engaged presenting part
- b) Direct mento-posterior position
- c) Fully dilated cervix
- d) Ruptured fetal membranes
- e) Empty urinary bladder

Which is the correct statement regarding obstetric forceps?

- a) Used only in vertex presentation
- b) Should be used before engagement of the fetal head
- c) Contraindicated with preterm birth
- d) Can be used during cesarean section
- e) May be used in brow malpresentation

Which is the incorrect statement regarding traction with the forceps?

- a) Traction should be intermittent
- b) Episiotomy is needed
- c) Delivery of fetal head should be slow
- d) Apply traction only during uterine contraction
- e) The fetal head should be extended

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Which of the following is not a prerequisite for a forceps delivery?

- a) Engagement of fetal head
- b) The occipito-anterior position
- c) A completely dilated cervix
- d) Ruptured membranes
- e) An empty bladder

Which is a contraindication of vacuum extractor?

- a) Fetal distress in the second stage
- b) Borderline cephalopelvic disproportion
- c) Shortening of the second stage in hypertensive mothers
- d) Suspected fetal coagulopathy
- e) Occipito-posterior position

Which is the incorrect statement regarding preoperative preparation for cesarean section?

- a) The bladder must be full to identify the lower uterine segment
- b) Intravenous fluids to keep normal hydration
- c) Skin area is prepared with antiseptic solution
- d) Blood transfusion is prepared for high risk pregnancies
- e) Confirm empty stomach

Which is the incorrect statement regarding anesthesia for cesarean section?

- a) Local anesthesia could be used in certain conditions
- b) Maternal position during the procedure is very important
- c) Anesthetic complications are significant cause of maternal mortality
- d) Postoperative pulmonary aspiration is a significant cause of maternal mortality
- e) Epidural anesthesia is associated with high risk of postoperative aspiration

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Which is the incorrect statement regarding postoperative care after uncomplicated cesarean section?

- a) Maintain intravenous fluids to avoid dehydration
- b) Monitor urine output
- c) Abdominal palpation to confirm uterine contraction
- d) Early ambulation is encouraged
- e) Breathing exercises are to be started only after the puerperium

Which is the incorrect statement regarding cesarean section?

- a) Safer than vaginal delivery for breech delivery
- b) Associated infection is markedly reduced by the use of prophylactic antibiotics
- c) Elective CS under regional anesthesia is safe for the mother as normal delivery
- d) Commonly performed for prolonged delivery
- e) The lower segment CS is safer than upper segment CS

Which is NOT an indication of cesarean section?

- a) Failed forceps delivery
- b) Previous two cesarean sections
- c) Fetal distress
- d) Old cervical tear
- e) Cord prolapse

Which is NOT an indication of cesarean delivery?

- a) Failure of trial of forceps
- b) Previous classic cesarean section
- c) Persistent late deceleration of the fetal heart rate in the first stage of labour
- d) Vaginal prolapse
- e) Prolapsed pulsating cord in the first stage of labour

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Which is NOT a cause for the current high rate of cesarean section?

- a) Increased prevalence of contracted pelvis
- b) Decreased chances for vaginal delivery of breech presentation
- c) Decreased chances for vaginal delivery in case of fetal distress
- d) Increased prevalence of women with previous cesarean delivery
- e) More safe anesthetic techniques

Why is the uterine scar following lower uterine segment CS stronger than that following upper segment CS?

- a) Proper surgical coaptation of the edges at repair of the upper segment incision
- b) Hematoma formation is less likely with the lower segment CS
- c) The upper segment is passive during puerperium
- d) The myometrium of the upper segment is stronger than that of the lower segment
- e) The upper segment incision is easier to repair

Which is the advantage to the upper segment cesarean section over the lower segment cesarean section?

- a) Less possibility of primary bleeding
- b) Less possibility of paralytic ileus
- c) Safer in the presence of pelvic adhesions
- d) Lower incidence of subsequent adhesions
- e) Lower possibility of infection

Which is NOT one of the prerequisites for vaginal birth after CS?

- a) Current singleton pregnancy
- b) Previous CS was LSCS and not USCS
- c) Current vertex presentation
- d) Previous only one CS
- e) Fetal weight not less than 4 kg in the current pregnancy

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Which is NOT an indication of Classical CS?

- a) Pelvic adhesions
- b) Anterior placenta previa
- c) Previous lower segment CS
- d) Neglected shoulder presentation
- e) Previous successful repair of vesico-vaginal fistula

Which is not an indication of cesarean hysterectomy?

- a) Uterine atony not responding to medical treatment
- b) Placenta accreta
- c) Uterine rupture that cannot be repaired
- d) Multiple fibroid uterus
- e) Bicornuate uterus

Which is the incorrect statement regarding obstetric forceps?

- a) It has traction action
- b) Cephalopelvic application is the safest application
- c) Low forceps is applied for station +2 or more
- d) Mid-forceps is applied for the unengaged head
- e) Can be applied for a malrotated head

Which is NOT a cause for failed forceps delivery?

- a) Contracted pelvis
- b) Uterine inertia
- c) Short umbilical cord
- d) Cephalopelvic disproportion
- e) Contraction ring

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Which is the **INCORRECT** statement regarding upper segment CS?

- a) Associated with increased blood loss
- b) Performed through a vertical incision in the lower uterine segment
- c) Indicated in previous successful repair of vesico-vaginal fistula
- d) Allows more rapid entry to the uterus than with the lower segment CS
- e) Associated with significant risk of subsequent uterine rupture

Which is an indication for emergency cesarean hysterectomy?

- a) Grand multiparous
- b) CIN
- c) Placenta circumvallate
- d) Placenta increta
- e) Red infarction of the placenta

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- True or False:

1. Mediolateral and median types of episiotomy have the same level of repair skills requirements
2. In Egypt the incidence of CS is 40%
3. Breech malpresentation is one of the absolute indications of CS
4. Selective CS is done during labor
5. Forceps may be applied for some delivery cases of face presentation
6. Cephalohematoma is more common with ventouse than with forceps delivery
7. Piper forceps has a longer pelvic curve than other forceps types
8. Upper segment CS can provide a faster access than lower segment CS for obstetric cases with fetal distress
9. There are two transverse skin incisions which can be used by the obstetrician for lower segment CS
10. The most common first fetal part seen during upper segment CS is fetal head
11. Ventouse can be used for operative vaginal delivery of fetus with gestational age of completed 37 weeks and vertex presentation

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12. The two recti muscles are incised transversely during lower segment CS.

13. Kielland forceps is used for outlet forceps delivery.

14. Pelvic curve of a single forceps blade is the concave part which comes in contact with fetal head.

15. The flexion point for ventouse cup application is at the occipital area.

16. The incidence of ruptured uterus in next pregnancy after upper segment CS is 0.4%.

17. CS is not contraindicated in a pregnant woman at 35 weeks' gestation with intrauterine fetal death and placenta previa centralis. She has previous two vaginal deliveries.

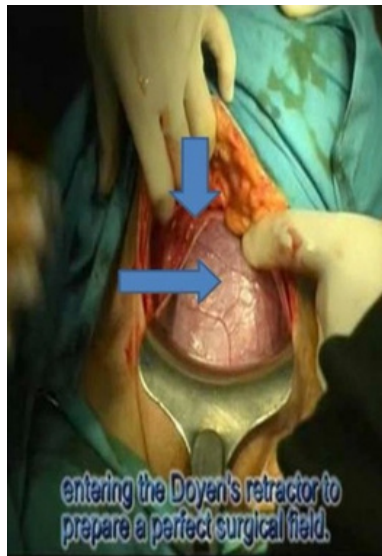
- Complete the Following Sentences

A. Postmortem cesarean section is an emergency C-section within (.....) minutes of the death of the mother in order to save the living fetus.

B. forceps may be used for delivery of after-coming head during breech-assisted vaginal delivery.

SUPPORT 41

- Mrs. O is a term pregnant woman undergoing a shown surgical procedure in attached image:



1. Name the procedure.

2. Name the structures referred to by arrows.

3. List four early complications of this procedure.

- Mrs. L is shown undergoing a certain operative obstetric procedure in attached image:



1. Name the procedure.

2. Name the structure referred to by the arrow.

3. Was the fetus delivered or not? Explain.

4. Name the incision done here and specify its criteria.

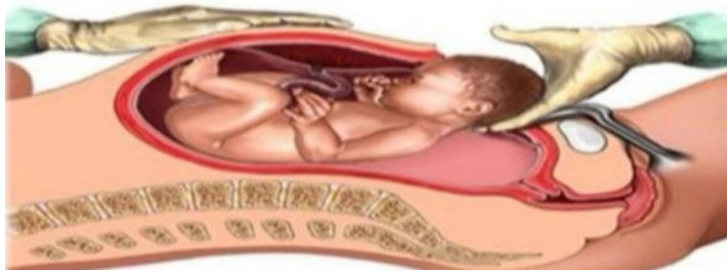
SUPPORT 41

- Mrs. F, a 23-year-old woman, has been scheduled for a certain surgical procedure which is shown done to her in the attached picture because of her 10 weeks missed miscarriage. The patient returned home to suffer a few hours later from recurrent vomiting, abdominal distension, and pain. Her temperature is 38.5°C.



1. What is the name of the procedure which was done for Mrs. F based on the shown image?
2. Name three types of anesthesia which can be utilized for such surgical procedure.
3. Name an instrument used for cervical dilatation.
4. What is the possible cause of Mrs. F's troubles based on the shown image and how to manage it?

SUPPORT 41



1. What is the name of this operation?
2. List its types according to timing.
3. Name the most common skin incision used for this operation.
4. List three late complications of this operation.



1. Name the shown procedure in the above image.
2. List three indications of the shown procedure.
3. State whether the following statements are correct or false:
 - a) The incidence of fetal cephalohematoma is lower with this procedure than forceps delivery ()
 - b) Traction of fetal head via shown procedure is via initiating positive pressure about 0.8 kilo per squared centimeter ()



- Regarding the shown image above:

1. What is the name of the shown instrument? And why are its blades fenestrated?
2. Name the parts of this instrument.
3. List three indications for its use.

SUPPORT 41

Which of the following respiratory functions is NOT increased during pregnancy?

- a) Tidal volume
- b) Ventilation
- c) PO_2
- d) Residual volume
- e) pH

Which of the following contributes to the increase in cardiac output in pregnancy?

- a) Decrease in blood volume
- b) Increase in ejection fraction
- c) Increase in heart rate
- d) Increase in left ventricular stroke work index
- e) Increase in systemic vascular resistance

All of the following are screening tests in the booking visit EXCEPT:

- a) Urine analysis
- b) Toxoplasmosis
- c) Rubella
- d) HBsAg
- e) 1-Hour postprandial blood glucose

All of the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal parts
- b) Auscultation of fetal heart sounds
- c) Uterine soufflé
- d) Umbilical soufflé
- e) Palpation of fetal movements

SUPPORT 41

Which of the following is a symptom of potentially serious pathology in late pregnancy?

- a) Swollen ankles
- b) Constipation
- c) Visual changes
- d) Nocturia
- e) Heartburn

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SUPPORT 41

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- e) Increase in systemic vascular resistance

Which methods of birth control needs a prescription?

- a) Birth control pill
- b) Cervical cap
- c) Diaphragm
- d) Male condom
- e) All of the above

How long is the vaginal ring left in place?

- a) 1 week
- b) 2 weeks
- c) 3 weeks
- d) 3 months

Which one of the following hormonal contraceptives can NOT be used during lactation?

- a) Pill - Mini
- b) Norplant
- c) DMPA
- d) Combined oral contraceptive pill

Combined OC pills:

- a) Reduce risk of endometrial cancer
- b) Increase risk of ovarian cancer
- c) Reduce risk of breast cancer
- d) Reduce risk of cervical cancer
- e) Worsen endometriosis

Fertility will most likely return earliest for:

- a) Nonbreastfeeding women postpartum
- b) Breastfeeding women postpartum
- c) Women postabortion

SUPPORT 41

The nurse is assessing the lochia on a 1-day PP patient. The nurse notes that the lochia is red and has a foul smelling odor. The nurse determines that this assessment finding is:

- a) Normal
- b) Indicates the presence of infection
- c) Indicates the need for increasing oral fluids
- d) Indicates the need for increasing ambulation

Which of the following findings would be a source of concern if noted during the assessment of a woman who is 12 hours postpartum?

- a) Postural hypotension
- b) Temperature of 100.4°F
- c) Pulse rate of 55 BPM - Bradycardia
- d) Pain in left calf with dorsiflexion of left foot

The first day of last menstrual period was August 30, 2021. So the expected date of delivery will be:

- a) May 31, 2022
- b) June 6, 2022
- c) June 17, 2022

Premarital care include all of the following EXCEPT:

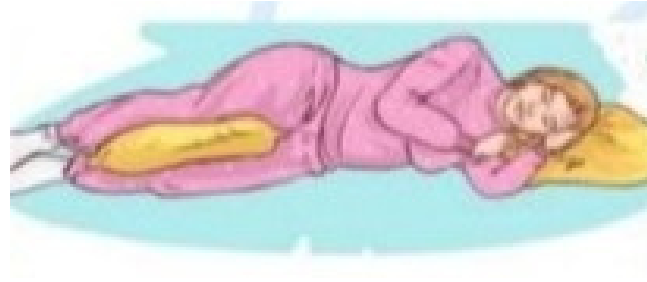
- a) Health promotion counselling
- b) Detection of health problem in each of them
- c) Immunization of each of them against rubella
- d) Reproductive health counselling

Genetic counselling includes which of the following (multiple answers):

- a) Family history of congenital diseases
- b) Prepare the couple for the possibility of having a child with special needs
- c) Referral for genetic testing
- d) Counselling about contraceptive methods to avoid having a child with congenital anomalies

SUPPORT 41

Which of the following is the best sleeping position during pregnancy?



When the woman comes late for one week for DMPA, you can give her the injectable?

- a) True
- b) False

All hormonal contraceptives have a negative effect on milk production?

- a) True
- b) False

Non-breastfeeding women postpartum should wait three months before beginning contraception.

- a) True
- b) False

Which of the following isn't a component of premarital care?

- a) Genetic counselling
- b) Promotion for proper ANC
- c) Basic Lab investigations
- d) Screening for substance abuse
- e) Control risk factors for high-risk pregnancy

When should inter-conception care be provided?

- a) Six month after the previous pregnancy
- b) During routine postpartum care
- c) Inter-conception care visits should be scheduled with the female during ANC visits
- d) During the child immunization visits

SUPPORT 41

Genetic counselling should be repeated during inter-conception care in the following conditions EXCEPT:

- a) There is history of recurrent abortion
- b) Consanguinity between parents
- c) Congenital abnormalities in the offspring
- d) Previous history of stillbirth
- e) History of previous child with thalassemia

Which of the following methods is the correct way to calculate the estimated date of delivery (EDD)?

- a) First day of last menstrual period (LMP) + 8 months + 1 week
- b) First day of last menstrual period (LMP) + 9 months
- c) Last day of last menstrual period (LMP) + 8 months + 1 week
- d) First day of last menstrual period (LMP) + 8 months
- e) First day of last menstrual period (LMP) + 9 months + 1 week

Your patient, 38 years old, asks you if she has to "hurry up" and get pregnant early after marriage. Regarding her age, which of the following is true?

- a) Advanced maternal age is not a contributor to infertility
- b) Pre-eclampsia and toxemia affect very young, usually nulliparous women and spare older women
- c) Diabetes mellitus is more commonly found in advanced maternal age
- d) The incidence of fetal anomalies increases with age, whereas the number of abortions decreases
- e) As long as your patient is in good health, exercises regularly, and does not have a chronic disease, postponing pregnancy till age of 40 is possible

SUPPORT 41

The primary purpose of prenatal care is to ensure a better outcome; a healthy baby and minimal risk to the mother. All the following are important to achieve this objective EXCEPT:

- a) Early, accurate assessment of gestational age
- b) Regular visits which focus on health and well-being of the mother
- c) Identification of the patient at risk (gestational diabetic, hypertensive)
- d) Performance of anatomic ultrasound in the second trimester
- e) Attention to emotional health of the patient and family

- A 35-year-old woman, on her first office visit, expresses her wish to become pregnant. She has only one child 5 years old, she denies any chronic health conditions.

1. Which of the following presents is NOT considered a risk factor for high-risk pregnancy?

- a) History of smoking with discontinuation 2 months ago
- b) History of domestic violence
- c) History of sexually transmitted diseases
- d) Family history of diabetes mellitus
- e) Previous history of stillbirth

2. During the physical examination for this lady, which of the following findings increase(s) the risk of the pregnancy?

- a) Elevated blood pressure
- b) A retroverted uterus
- c) A thrombosed hemorrhoid
- d) Obesity
- e) Detected murmur of mitral stenosis

SUPPORT 41

During ANC for a 27-year-old female in her 1st pregnancy, she indicates that she is concerned about "getting fat" during pregnancy. She is also concerned about breastfeeding for the same reason because people have told her that she would "have to eat for the baby" if she breastfeeds. She is 155 cm tall and has a body mass index (BMI) of 20. How much weight gain do you recommend?

- a) 8 to 11.5 kg
- b) 12.5 to 18 kg
- c) 11.5 to 16 kg
- d) 5 to 9 kg
- e) None of the above

In counseling your patient on weight gain, which of the following is NOT a complication associated with excessive weight gain?

- a) Gestational HTN
- b) Gestational diabetes
- c) Shoulder dystocia
- d) Intrauterine growth retardation
- e) Post-pregnancy obesity

At what stage of gestation would you expect a multiparous woman to begin to feel fetal movements?

- a) At 12 weeks
- b) At 16 weeks
- c) At 20 weeks
- d) At 22 weeks
- e) 14 weeks

SUPPORT 41

Which of the following statements regarding alcohol consumption in pregnancy and fetal alcohol syndrome is/are true?

- a) Limit alcohol intake to one or two glasses of wine
- b) Abstain completely from alcohol during pregnancy
- c) Children with fetal alcohol syndrome have developmental problems, behavioral problems and learning difficulties
- d) Alcohol consumption in the third trimester is not directly associated with fetal alcohol syndrome
- e) Children with fetal alcohol syndrome have major facial features

A 42-year-old pregnant woman was very worried during ANC visit that she has missed the right time to have her combined test for Down's syndrome screening. She is now 17 weeks pregnant. You counsel her about the quadruple test and arrange to have this done. Which of the following is/are true about quad test?

- a) The best time for quad test from 16–18 weeks of gestation
- b) Unconjugated oestradiol, hCG, AFP and inhibin A
- c) Beta hCG, AFP, nuchal translucency and inhibin A
- d) AFP, inhibin B, beta hCG and oestradiol
- e) Can be done from 15–20 weeks of gestation

Which of the following is NOT related to intrauterine growth restriction (IUGR)?

- a) Hypertension
- b) Smoking
- c) Hypercholesterolemia
- d) Pre-eclampsia
- e) Diabetes mellitus

SUPPORT 41

- True or False

A 37-year-old pregnant lady came for her 1st antenatal care visit. She has been trying to get pregnant for 9 years. She suffered from PCOS and irregular menstruation. Her last menses was in 6th January. She has a +ve home pregnancy test.

1. Her EDD will be 13th October 2022.

2. This lady is considered as having a high risk pregnancy.

3. U/S is mandatory in the 1st ANC visit for this lady.

- Enumerate:

1. Advanced maternal age is a leading cause of what?

2. The effects of alcohol consumption during pregnancy.

3. Values of U/S examination.

4. Ultrasound dating.

- A young lady, 25 years old, teacher in primary school, visits the clinic to ask about services that could be provided for her before marriage. She is planning for marriage within 4 months:

1. List services that you can offer her at this time

2. List three important items in history taking during premarital screening

3. List routine investigations done in premarital screening

SUPPORT 41

- What are the counseling sessions provided for the couples during premarital care?
- What is the importance of ANC?
- A woman comes in for counseling who has just had an uncomplicated first trimester abortion. Check all the contraceptive methods she can use immediately
- What are components of antenatal care?
- List proper timings for ANC.
- List two sure signs of pregnancy.
- Enumerate important physical examination during repeated ANC visits.
- List important educational messages for the pregnant female.

SUPPORT 41

A woman complains of right sided abdominal pain and mild vaginal bleeding for one day after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. There is tenderness in the right iliac fossa. The urine pregnancy test is positive. Transvaginal ultrasound scan does not reveal any adnexal mass or intrauterine pregnancy. What is the next step of management?

- a) Repeat ultrasound scan after 7 days
- b) Start treatment with methotrexate
- c) Perform two serum beta HCG tests 46 hours apart
- d) Perform laparoscopic examination

Primigravida with full term complains of fainting on lying down and she feels well when she turns to one side. This is due to:

- a) Increased abdominal pressure
- b) Inferior vena cava compression
- c) Increased intracranial pressure
- d) After heavy lunch

A 30-year-old P2+2 female has been diagnosed to have complete vesicular mole. The treatment of choice would be:

- a) Total hysterectomy
- b) Suction evacuation
- c) Radiotherapy
- d) Expectant management

All the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal parts
- b) Auscultation of FHS
- c) Umbilical soufflé
- d) Uterine soufflé

SUPPORT 41

Human Chorionic Gonadotropin (HCG) is produced by:

- a) Amniotic membranes
- b) Fetal yolk sac
- c) Hypothalamus
- d) Trophoblastic cells

A 28-year-old female with history of 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound examination there is gestational sac with absent fetal parts. The diagnosis is:

- a) Corpus luteum cyst
- b) Ectopic pregnancy
- c) Threatened abortion
- d) Blighted ovum

With regards to acute pyelonephritis with pregnancy, all are true EXCEPT:

- a) More common in second trimester of pregnancy
- b) Most common isolate is E. coli
- c) Responds to cephalosporins
- d) The incidence of left kidney involvement is more than the right kidney

A patient of 36 weeks gestation presents with hypertension, blurring of vision and headache. Her blood pressure reading was persistent 180/120. Proteinuria +3. How will you manage this patient:

- a) Start antihypertensive and follow up in outpatient department
- b) Admit the patient, start antihypertensive and continue pregnancy till term
- c) Admit the patient and observe
- d) Admit the patient, start antihypertensive, MgSO_4 and terminate the pregnancy

SUPPORT 41

In a pregnant woman with heart disease, all the following are to be done EXCEPT:

- a) IV methergin after delivery
- b) Prophylactic antibiotic
- c) IV furosemide postpartum
- d) Shortening second stage of labor

Uterine blood flow at term is:

- a) 750 ml/min
- b) 250 ml/min
- c) 350 ml/min
- d) 50 ml/min

A woman complains of right sided lower abdominal pain and mild vaginal bleeding after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. Serum HCG is 2200 IU/ml. Transvaginal ultrasound reveals adnexal mass 20 mm with no cardiac activity, and no intrauterine pregnancy. The most appropriate management is:

- a) Perform laparoscopic salpingostomy
- b) Treat with intramuscular methotrexate
- c) Treat with intramuscular methyl ergometrine
- d) Perform laparoscopic salpingectomy

A 25-year-old female, Gravida 5, Para 0+3, with history of recurrent early pregnancy loss. Which of the following investigations is NOT ordered?

- a) Thyroid function test
- b) Testing for antiphospholipid syndrome
- c) Testing for TORCH
- d) Genetic testing and karyotyping

SUPPORT 41

Regarding the process of fertilization, which of the following statements is correct?

- a) Fertilization occurs most commonly in isthmus of the fallopian tube
- b) Implantation occurs most commonly on posterior wall near the fundus
- c) Implantation begins 14 days after fertilization
- d) Morula enters uterus on day 7

All of the following are maternal changes during pregnancy EXCEPT:

- a) Increased fibrinogen
- b) Increased cardiac output
- c) Increased glomerular filtration rate
- d) Increased alveolar residual volume

Which of the following Leopold's maneuvers is shown in the image:

- a) Umbilical grip
- b) Second pelvic grip
- c) Fundal grip
- d) First pelvic grip



The following are related to uterine soufflé EXCEPT:

- a) It can be heard in a big fibroid
- b) It is due to increased blood flow through the umbilical artery
- c) The sound is synchronous with maternal pulse
- d) It is a soft blowing systolic murmur heard on the sides of the pregnant uterus

Identify the placental abnormality shown in the figure:

- a) Succenturiate placenta
- b) Placenta membranacea
- c) Bi-lobed placenta
- d) Velamentous insertion of the cord



SUPPORT 41

A 42-year-old female, P4+2, has been diagnosed with complete vesicular mole. The treatment of choice would be:

- a) Chemo radiotherapy
- b) Hysterectomy
- c) Chemotherapy
- d) Radiotherapy

A Primigravida, 32 years old female, admitted to the antenatal ward at 35 weeks. Her BP was 160/110. Her urine does not contain albumin. Which of the following drugs should NOT be used in her management?

- a) Methyldopa
- b) Furosemide
- c) Labetalol
- d) Hydralazine

The most common rheumatic heart disease associated with pregnancy is:

- a) Mitral regurgitation
- b) Mitral stenosis
- c) Aortic regurgitation
- d) Aortic stenosis

All of the following are true regarding cephalohematoma EXCEPT:

- a) Develops hours after birth
- b) Well-defined edges
- c) Normal overlying skin
- d) Disappears within two days
- e) Localized over one bone

G2 Para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Fetal distress
- b) Fresh vaginal bleeding
- c) Severe continuous abdominal pain
- d) Maternal tachycardia

SUPPORT 41

The uterus becomes pelvic organ after delivery by:

- a) Four weeks
- b) Two weeks
- c) Three weeks

Ventouse in second stage of labor is contraindicated in:

- a) Persistent occipito-posterior position
- b) Uterine inertia
- c) Heart disease
- d) Preterm labor

Lochia in correct order during puerperium is:

- a) Alba, mucosa, serosa
- b) Alba, serosa, rubra
- c) Rubra, serosa, alba
- d) Serosa, rubra, alba

All of the following are true for episiotomy EXCEPT:

- a) Involvement of the anal sphincter is classified as 4th degree perineal tear
- b) Midline episiotomy bleeds less, easier to repair, and heals more quickly
- c) Can be either midline or mediolateral
- d) Allows widening of the birth canal

The nerve roots affected in Klumpke's palsy are:

- a) C7, C8, T1
- b) C8, T1
- c) C1, C2, T1
- d) C7, C8, T2
- e) C5, C6

SUPPORT 41

Forceps delivery can be done in all EXCEPT:

- a) Maternal heart disease
- b) After-coming head in breech delivery
- c) Deep transverse arrest
- d) Persistent mento-posterior

Regarding indications of cesarean hysterectomy, which of the following statements is NOT true?

- a) Placenta accreta encountered during cesarean section
- b) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- c) The mere presence of Couvelaire uterus
- d) Uncontrolled postpartum hemorrhage
- e) Operable carcinoma of the cervix during pregnancy

Prerequisites of outlet forceps include all the following EXCEPT:

- a) Uterus contracting
- b) Engaged head
- c) Station 0 or +1
- d) Fully dilated cervix

A hypertensive pregnant woman at 34 weeks comes with history of abdominal pain, vaginal bleeding and loss of fetal movement. On examination, the uterus is contracted with increased uterine tone. Fetal heart rate is absent. The most likely diagnosis is:

- a) Abruptio placenta
- b) Placenta previa
- c) Premature rupture of membrane
- d) Vasa previa

SUPPORT 41

The placenta of twins can be:

- a) Di-chorionic and mono-amniotic in dizygotic twins
- b) Di-chorionic and mono-amniotic in monozygotic twins
- c) Mono-chorionic and mono-amniotic in dizygotic twins
- d) Mono-chorionic and di-amniotic in monozygotic twins

This image shows one of the maneuvers used in shoulder dystocia. This maneuver is:

- a) Screw maneuver
- b) Internal rotation
- c) Delivery of posterior arm
- d) Zavanelli maneuver



A 35-year-old woman, G2 P1+0, presents to antenatal clinic at 35 weeks of pregnancy with sudden gush of watery vaginal discharge. Sample of pooled liquid turned red litmus paper blue and ferning was present. The temperature of the patient was 38°C and her pulse was 102 per minute.

What is the next step in management?

- a) Administer antibiotics
- b) Place cervical cerclage
- c) Administer betamethasone
- d) Administer tocolytics

Primigravida at 37 weeks gestation reported to labor room with central placenta previa with heavy vaginal bleeding. The fetal heart rate was normal at the time of examination. The best management option for this patient is:

- a) Expectant management
- b) Induction and vaginal delivery
- c) Cesarean section
- d) Induction and forceps delivery

SUPPORT 41

All are causes of intrauterine growth restriction EXCEPT:

- a) Pregnancy-induced hypertension
- b) Tubal ectopic pregnancy
- c) Maternal heart disease
- d) Anemia

A pregnant woman is found to have excessive accumulation of amniotic fluid. Such polyhydramnios is likely to be associated with all of the following conditions EXCEPT:

- a) Twins pregnancy
- b) Bilateral renal agenesis
- c) Microcephaly
- d) Esophageal atresia

All of the following are contraindications to tocolysis EXCEPT:

- a) Anencephaly
- b) Placenta previa
- c) Fetal distress
- d) Chorioamnionitis

Complications of diabetes in pregnancy include all the following EXCEPT:

- a) Neural tube defect
- b) IUGR
- c) Shoulder dystocia
- d) Hyperglycemia in newborn
- e) Macrosomia

A Gravida 1, Para 0+0 woman, attends antenatal clinic for the booking visit at 12 weeks. Her blood group is B rhesus negative. What is the next step in management:

- a) Perform Rhesus antibody titre
- b) Determine the husband's blood group
- c) Perform ultrasound scan
- d) Determine fetal blood group

SUPPORT 41

Techniques of delivery of after-coming head in breech presentation include all EXCEPT:

- a) Piper's forceps
- b) External cephalic version
- c) Burns-Marshall's technique
- d) Jaw flexion shoulder traction

The smallest diameter of true pelvis is:

- a) Diagonal conjugate diameter
- b) True conjugate diameter
- c) Intertuberous diameter
- d) Interspinous diameter

A woman undergoes induction of labor with amniotomy followed by oxytocin infusion at a period of amenorrhea 42 weeks. Six hours later she is getting 4 contractions per 10 minutes and the fetal heart rate drops to 100 per minute. What is the first step in management?

- a) Stop oxytocin infusion
- b) Perform cardiotocograph
- c) Change the position of the mother
- d) Administer oxygen to the mother

In which fetal presentation can vaginal delivery be expected?

- a) Shoulder presentation
- b) Transverse lie
- c) Brow presentation
- d) Face presentation when the chin lies under the symphysis pubis

Cesarean section is mandatory in the following presentation:

- a) Face
- b) Cephalic
- c) Brow
- d) Vertex

SUPPORT 41

The most common cause of postpartum hemorrhage is:

- a) Trauma
- b) Retained placental products
- c) Uterine atony
- d) Coagulopathy

In pathological retraction ring, all are true EXCEPT:

- a) Indicates obstructed labour and needs cesarean section
- b) Appears as a visible and palpable groove rising across the abdomen
- c) Can be felt vaginally
- d) More common in multiparous women

A female bleeds profusely soon after delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ecobolics. Her HR = 120/min, BP = 90/60. What is the next appropriate step of management?

- a) Ligate internal iliac artery
- b) Do hysterectomy
- c) Apply B-Lynch suture
- d) Carry out balloon tamponade
- e) Ligate uterine arteries

Mrs. H, Gravida 2 Para 1+0, presented to hospital in labor pains. On examination, she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 6 cm and FHR 145 per minute. What is the stage of labor?

- a) Stage III
- b) Stage II
- c) Stage V
- d) Stage IV
- e) Stage I

SUPPORT 41

Abnormal uterine action includes all of the following EXCEPT:

- a) Constriction ring
- b) Cervical insufficiency
- c) Colicky uterus
- d) Pathological retraction ring
- e) Cervical dystocia

SUPPORT 41

All the following can pass through the placental barrier EXCEPT:

- a) Rubella
- b) Treponema pallidum
- c) Heparin
- d) Antibiotics
- e) Hormones

All of the following are characteristics of HELLP syndrome EXCEPT:

- a) Anemia
- b) Increased SGOT and SGPT
- c) Hemolysis
- d) Thrombocytopenia
- e) Hyperuricemia

Patient with hemoglobin 6 gm% at 38 weeks gestation is admitted in the antenatal ward. If the anemia is hypochromic and microcytic, the most appropriate management of the patient is:

- a) Blood transfusion to raise hemoglobin to 9 gm%
- b) Administration of intravenous iron so that anemia can be corrected more rapidly
- c) Double the dose of her oral iron intake
- d) Administration of intramuscular or intravenous iron depending on her pulse rate
- e) Induction of labor immediately so that further deterioration of anemia is prevented

Which of the following endometrial histopathological findings is suggestive of ectopic pregnancy?

- a) Chorionic villi
- b) Endometrial hyperplasia
- c) Secretory endometrium
- d) Decidual reaction
- e) Proliferative endometrium

SUPPORT 41

In case of diabetes complicating pregnancy:

- a) Insulin requirement decreases as pregnancy advances
- b) The best screening test is urine glucose
- c) Congenital malformations are unrelated to diabetic control
- d) Oral hypoglycemics are preferred to insulin therapy
- e) There is increased incidence of hydramnios and preeclampsia

A woman who is nine weeks pregnant comes to the early pregnancy assessment unit complaining of severe nausea and occasional vomiting. She is not keen on drug therapy. What is your advice?

- a) Drink decaffeinated coffee
- b) Drink herbal tea
- c) Take a long break from work
- d) Take up yoga
- e) Drink ginger syrup

What dose of glucose has been recommended by WHO for oral glucose tolerance test during pregnancy?

- a) 100 gm
- b) 150 gm
- c) 200 gm
- d) 50 gm
- e) 75 gm

Antidote of high dose heparin therapy for thromboembolism during pregnancy is:

- a) Diazepam
- b) Protamine sulphate
- c) Fresh frozen plasma
- d) Vitamin K
- e) Calcium gluconate

SUPPORT 41

All of the following are sure signs of pregnancy EXCEPT:

- a) Umbilical soufflé
- b) Palpation of fetal movement
- c) Quickening
- d) Auscultation of fetal heart sound
- e) Palpation of fetal parts

The following are physiological changes of thyroid gland during pregnancy EXCEPT:

- a) Increased protein bound iodine
- b) Decreased basal metabolic rate
- c) Increased T4
- d) Slight diffuse enlargement of thyroid gland
- e) Increased T3

All of the following are ultrasound features of intrauterine fetal demise EXCEPT:

- a) Turtle sign
- b) Spalding sign
- c) Collapse of the thorax
- d) Absent fetal movement
- e) Gases in the intestine

All of the following are sequelae of macrosomic baby EXCEPT:

- a) Shoulder dystocia
- b) Erb's palsy
- c) Obstructed labor
- d) Meconium aspiration syndrome
- e) Disseminated intravascular coagulopathy

SUPPORT 41

The following abnormal findings can be found in IUGR baby EXCEPT:

- a) Absent uterine artery diastolic notch
- b) Change in Head/Abdomen ratio
- c) Absent umbilical artery diastolic blood flow
- d) Oligohydramnios
- e) Increased maternal serum alpha fetoprotein

A woman is admitted to hospital with history of dribbling for 24 hours at a period of amenorrhea of 32 weeks. Speculum examination confirms dribbling. The liquor is offensive. Her temperature is 38°C. High reactive protein. No uterine contractions. FHR is 170 per minute. What is the most appropriate treatment?

- a) Perform cesarean section immediately after administering 1 dose of corticosteroids and broad spectrum antibiotics
- b) Augment labor with intravenous infusion of oxytocin
- c) Administer broad spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- d) Repeat C-reactive protein 24 hours after commencing broad spectrum antibiotics

A primigravida complains of lower abdominal pain at a period of amenorrhea of 32 weeks. The fetus is in cephalic presentation. She has 1–2 uterine contractions per 10 minutes. Cervix is 2 cm dilated and 75% effaced with intact membranes. What is the preferred next step of management?

- a) Start intramuscular corticosteroids and intravenous ritodrine
- b) Start intravenous broad spectrum antibiotics
- c) Commence intramuscular corticosteroids and oral nifedipine
- d) Allow labor to progress normally
- e) Commence intravenous ecobolics

SUPPORT 41

Monozygotic twins have the following characteristics EXCEPT:

- a) Has different blood groups
- b) Accounts for 20% of twins pregnancy
- c) More susceptible to twin-to-twin transfusion syndrome
- d) Has different fingerprints
- e) Has the same gender

All of the following maneuvers are performed in shoulder dystocia EXCEPT:

- a) Delivery of posterior arm
- b) McRoberts' position
- c) Fundal pressure
- d) Episiotomy
- e) Rotation of anterior shoulder

The following vaccines are safe during pregnancy EXCEPT:

- a) Hepatitis B
- b) Pneumococcus
- c) Rubella
- d) Tetanus
- e) Influenza

Which of the following diseases can cause polyhydramnios:

- a) IUGR
- b) Absent kidneys
- c) Placental insufficiency
- d) Duodenal atresia
- e) Post maturity

SUPPORT 41

With reference to fetal heart rate, a non-stress test is considered reactive when:

- a) Two fetal heart rate accelerations are noted in 10 minutes
- b) Fetal heart accelerations during uterine contractions
- c) One fetal heart rate acceleration is noted in 20 minutes
- d) Three heart rate accelerations are noted in 30 minutes
- e) Two fetal heart rate accelerations are noted in 20 minutes

SUPPORT 41

A 22 years old female underwent suction evacuation for molar pregnancy. Beta HCG levels are persistently high 3 months following evacuation. The next step of management is:

- a. Follow up at 1 year
- b. Insertion of intrauterine device (IUD)
- c. Methotrexate
- d. Hysterectomy
- e. Monitor till HCG is zero

All of the following are structures of the fetal membranes except:

- a. Decidua
- b. Allantois
- c. Yolk sac
- d. Connecting stalk
- e. Chorion

In case of diabetes mellitus with pregnancy:

- a. The best screening test is urine glucose
- b. Oral hypoglycemics are preferred to insulin therapy
- c. Insulin requirement decreases as pregnancy advances
- d. Congenital malformations are unrelated to diabetic control
- e. There is increased incidence of hydramnios and preeclampsia

The time at which cardiac output is highest is during which phase of pregnancy?

- a. In the first trimester
- b. In the second trimester
- c. Immediately postpartum
- d. During labor
- e. In the third trimester

SUPPORT 41

A woman presents with abdominal pain and nausea with amenorrhea 5 weeks. Ectopic pregnancy is diagnosed if:

- a. Beta HCG more than 2000 IU/l with no gestational sac in the uterus by transvaginal US
- b. Beta HCG less than 1000 IU/l with empty uterus on transvaginal US
- c. Serum progesterone is more than 25 ng/dl
- d. Beta HCG more than 1000 IU/l with endometrial thickness of 14 mm
- e. Serum prolactin more than 20 ng/dl

All of the physiological changes during pregnancy except:

- a. Increased fibrinogen
- b. Decreased thyroxin binding globulin
- c. Increased tidal volume
- d. Increased glomerular filtration rate
- e. Increased stroke volume

A primigravida woman with mitral stenosis with no pulmonary hypertension is in labor at 39 weeks. She has dyspnea only in severe exertion. Fetus is in cephalic presentation. Cervix dilatation is 5 cm. She has two uterine contractions in 10 minutes. Which of the following should be avoided in this patient:

- a. Place the patient in semi sitting position
- b. Ventouse delivery in the second stage of labor if delivery does not occur in half hour
- c. Use of epidural analgesia for pain relief
- d. Active management of third stage of labor
- e. Use of ergometrine in the third stage of labor

SUPPORT 41

A woman is admitted to the antenatal ward at 35 weeks. Her blood pressure is 179/110 and the urine contains albumin. She has generalized seizure during examination. An oropharyngeal airway is inserted. Which of the following is the most appropriate step in management?

- a. Immediate transfer the patient for cesarean section
- b. Insert an indwelling catheter
- c. Give 10 mg of intravenous hydralazine
- d. Admit to the intensive care unit
- e. Give 4 grams bolus of magnesium sulphate

A primigravida presents in emergency room with painless bleeding at 12 weeks of gestation. Cervix is closed and normal. What is the most probable diagnosis?

- a. Incomplete abortion
- b. Placenta previa
- c. Inevitable abortion
- d. Septic abortion
- e. Threatened abortion

Your patient has microcytic anemia with a hemoglobin of 9 and normal iron stores. What is the most likely diagnosis?

- a. Iron deficiency anemia
- b. Folate deficiency
- c. Vitamin B6 deficiency
- d. Thalassemia
- e. Vitamin B12 deficiency

All of the following are components of biophysical profile EXCEPT:

- a. Non stress test
- b. Fetal body movement
- c. Fetal respiratory movement
- d. Oxytocin challenge test
- e. Amniotic fluid volume

SUPPORT 41

A woman with dichorionic diamniotic twins attends antenatal clinic at 36 weeks. The first twin is in breech presentation and the second is in cephalic presentation. She has no other pregnancy complications. What is the most appropriate management?

- a. Await spontaneous onset of labor
- b. Induce labor at 37 weeks
- c. Perform internal podalic version
- d. Perform external cephalic version of the first twin
- e. Perform elective cesarean section at 37 weeks

The pathognomonic sign of monochorionic diamniotic twins pregnancy by ultrasonography is:

- a. Y sign
- b. V sign
- c. U sign
- d. Lamda sign
- e. T sign

A primipara whose period of amenorrhea is 39 weeks develops sudden severe abdominal pain, shock and tender uterus without vaginal bleeding. She is not in labor. What is the most likely diagnosis?

- a. Rupture uterus
- b. Abruptio placenta
- c. Amniotic fluid embolism
- d. Chorioamnionitis
- e. Red degeneration of fibroid

All predispose to isoimmunization in Rh negative female EXCEPT:

- a. Ectopic pregnancy
- b. Advanced maternal age
- c. Vaginal delivery
- d. Antepartum hemorrhage
- e. Cesarean section

SUPPORT 41

A woman is admitted at 36 weeks gestation following sudden episode of antepartum hemorrhage. The bleeding has ceased on admission. She is not pale. BP 120/80 and pulse is 76 beats/minute. The uterus is soft and not tender. There are no palpable uterine contractions. FHR is 140/minute. What is next step of management?

- a. Administer intravenous dexamethasone
- b. Examine the cervix using Cusco's speculum
- c. Pelvic examination under general anesthesia
- d. Ultrasound examination of the uterus
- e. Perform cardiotocograph

A woman complains of sudden onset of watery vaginal discharge at a period of amenorrhea 32 weeks. The fetus is in cephalic presentation with no uterine contractions. FHR 140 beats per minute. What is the first step in management?

- a. Perform ultrasound scan to assess amniotic fluid volume
- b. Commence intravenous corticosteroids
- c. Commence intravenous tocolysis
- d. Commence broad spectrum antibiotics
- e. Perform sterile speculum examination

A 25 year-old lady at 39 weeks of gestation, during labor, a hand is found beside the head in cephalic presentation, what is your diagnosis?

- a. Brow presentation
- b. Occipito-posterior position
- c. Compound presentation
- d. Occipito-anterior position
- e. Face presentation

SUPPORT 41

The following techniques are used for delivery of the after-coming head in breech presentation EXCEPT:

- a. Mauriceau Smellie Veit technique
- b. Forceps delivery
- c. Burns Marshal technique
- d. Lovset maneuver
- e. Jaw flexion shoulder traction

Which is not included in the management of the third stage of labor:

- a. Uterotonic within one minute of delivery
- b. Massage of the uterus
- c. Direct oxytocin injection after delivery of the shoulders
- d. Immediate cutting and cord clamping
- e. Controlled and sustained cord traction

True about frank breech:

- a. Thigh flexed, knee extended
- b. More common in multipara
- c. Both are flexed
- d. Buddha attitude
- e. Thigh extended, knee extended

Treatment of postpartum hemorrhage is all EXCEPT:

- a. Carbetocin
- b. Syntometrine
- c. Oestrogen
- d. Oxytocin

What is the denominator of face presentation?

- a. Fetal nose
- b. Frontal bone
- c. Chin
- d. Orbital margin
- e. Fetal mouth

SUPPORT 41

Ritgen maneuver is done in:

- a. For delivery of the head in normal labor
- b. For delivery of the head in breech presentation
- c. Shoulder dystocia
- d. For delivery of the head in brow presentation
- e. For delivery of the legs in breech presentation

In which presentation the fetal head is deflexed:

- a. Occipito-anterior position
- b. Brow presentation
- c. Occipito-posterior position
- d. Compound presentation

What is the measurement of sub-occipito-bregmatic diameter?

- a. 11 cm
- b. 10 cm
- c. 10.5 cm
- d. 9.5 cm
- e. 9 cm

All of the following respiratory functions are increased during pregnancy EXCEPT:

- a. Tidal volume
- b. Ventilation
- c. PO₂
- d. Residual volume
- e. PH

The most worrisome sign or symptom of potentially serious pathology in late pregnancy is which of the following?

- a. Swollen ankles
- b. Constipation
- c. Visual changes
- d. Nocturia
- e. heartburn

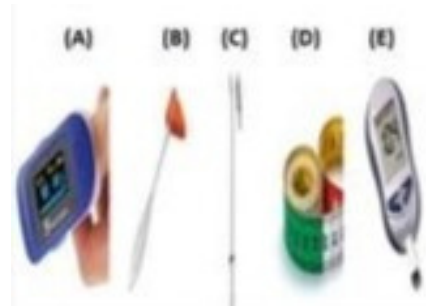
SUPPORT 41

How much is the incidence of hypertensive disorders in pregnancy?

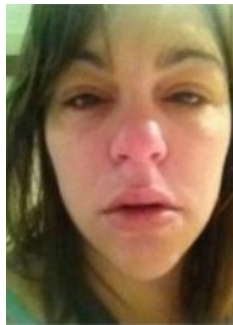
- a) 12 - 20 %
- b) 5 - 8 %
- c) 40 - 45%
- d) 25 - 35%

Which of the following tools is used to check for unwanted high magnesium levels during magnesium sulphate infusion as anticonvulsant prophylaxis in a preeclamptic patient?

- a) Tool (B)
- b) Tool (A)
- c) Tool (D)
- d) Tool (E)
- e) Tool (C)



Mrs. K, a 40-year-old primigravida at 31 weeks' gestation, presents with frontal headache for two days. Her blood pressure is 165/110 mmHg and urinary analysis shows 3+ proteinuria. CBC shows hemoglobin 11.4 gm/dl and platelet count 70,000/cmm. AST is 74 IU/L and ALT is 90 IU/L. What is the best management of this woman?



- a) Start magnesium sulphate infusion and follow up the patient
- b) Regular close frequent follow up as her condition may subside gradually
- c) Control her blood pressure and follow up the patient
- d) Blood transfusion and platelets transfusion
- e) Preparation for termination of pregnancy after control of blood pressure and using magnesium sulphate infusion

SUPPORT 41

Which of the following is not a risk factor for preeclampsia?

- a) Multiparity
- b) Hydatidiform mole
- c) Obesity
- d) Diabetes mellitus
- e) Chronic renal impairment

A 30-year-old multiparous woman attends the clinic at 12 weeks of her third pregnancy. She had recurrent preeclampsia in two previous pregnancies. Her blood pressure is 110/70 mmHg with negative proteinuria, and BMI is 30 kg/m². What is the correct management?

- a) Start alpha methyl dopa as a prophylactic medication
- b) Measure the blood pressure again after 6 hours in a quiet environment
- c) Careful antenatal care and start oral low dose aspirin
- d) Refer the patient for an internal physician to manage her blood pressure
- e) Ask the patient to come again to the clinic after one week

Which of the following is not a major threat to maternal life for a woman with preeclampsia?

- a) Placental insufficiency
- b) Stroke
- c) Placental abruption
- d) Hepatic hemorrhage
- e) Eclampsia

SUPPORT 41

A 29-year-old woman, G2P2, is in the postpartum period after full-term spontaneous vaginal delivery. She had severe preeclampsia with BP >163/110 mmHg and proteinuria of 5.5 g/24hr. Magnesium sulfate infusion was started. After 8 hours, her respiratory rate is 11/min, knee jerks are absent, and urine output is 60 ml over 4 hours. What is the best management?

- a) Lower magnesium sulfate infusion rate and inject 2 gm of calcium gluconate intravenously over 6 minutes
- b) Stop magnesium sulfate infusion and apply oxygen mask
- c) Close follow up with continuing magnesium sulfate infusion
- d) Stop magnesium sulfate infusion and inject 1 gm of calcium gluconate intravenously over 3 minutes
- e) Lower magnesium sulfate infusion rate with close follow up

A 24-year-old woman comes to the clinic for the first time at 32 weeks of gestation in her first pregnancy. Her BP is 140/90 mmHg, she feels well, and has no history of chronic hypertension. What is the correct management?

- a) Ask the patient to follow up at antenatal clinic after two weeks
- b) Tell the patient that she is hypertensive and counsel her for possible developing obstetric complications
- c) Measure the blood pressure again after 4–6 hours in a quiet environment
- d) Advise the patient to avoid salty food
- e) Start oral alpha methyl dopa or labetalol

SUPPORT 41

A 21-year-old pregnant woman at 35 weeks complains of headache and epigastric pain for 3 days. BP 140/90 mmHg, proteinuria 3+, reflexes exaggerated, platelets $95,000/\text{mm}^3$, elevated liver enzymes, fetus breech. What is the correct management?

- a) Start magnesium sulphate infusion with close follow up
- b) Measure the blood pressure again after 6 hours in a quiet environment
- c) Start oral alpha methyl dopa or labetalol
- d) Termination of pregnancy by cesarean section after starting IV magnesium sulfate
- e) Wait for another two weeks under good antihypertensive control

A 40-year-old multiparous lady at 10 weeks gestation has BP 160/102 mmHg, ankle edema, proteinuria 106 mg/dl, and history of essential hypertension. What is the best management?

- a) Start oral alpha methyl dopa or labetalol and low oral dose of aspirin
- b) Termination of pregnancy by induction of abortion
- c) Close clinic follow up with the use of oral magnesium supplementation
- d) Measure the blood pressure again after 6 hours in a quiet environment
- e) Start preferably beta blockers type of antihypertensive medications

SUPPORT 41

All of the following are screening tests in the booking visit EXCEPT:

- a) Urine analysis
- b) Toxoplasmosis
- c) Rubella
- d) HBsAg
- e) 1-Hour postprandial blood glucose

All of the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal parts
- b) Uterine soufflé
- c) Auscultation of fetal heart sounds
- d) Umbilical soufflé
- e) Palpation of fetal movements

Which of the following factors contribute to increase in cardiac output in pregnancy?

- a) Decrease in blood volume
- b) Increase in ejection fraction
- c) Increase in heart rate
- d) Increase in left ventricular stroke work index
- e) Increase in systemic vascular resistance

Which of the following is not an absolute indication for cesarean delivery?

- a) Cervical fibroid measuring 20 cm
- b) Major degree of placenta previa associated with an intrauterine fetal death
- c) Fetal macrosomia
- d) When the true conjugate diameter of maternal pelvis is less than 6 cm
- e) Conjoined twins

SUPPORT 41

Which of the following is NOT an early complication of cesarean delivery?

- a) Allergic reaction to anesthetic agents
- b) Total intra-operative blood loss of 580 ml
- c) Amniotic fluid embolism
- d) Uterine artery injury
- e) Bladder injury

Regarding indications of cesarean hysterectomy, which of the following statements is NOT true?

- a) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- b) Operable carcinoma of the cervix during pregnancy
- c) Uncontrolled postpartum hemorrhage
- d) Placenta accreta encountered during cesarean section
- e) The mere presence of couvelaire uterus

Which of the following is NOT an indication for forceps assisted vaginal delivery?

- a) A second stage more than 190 minutes
- b) A fetal heart rate of 60 beats per minute during the second stage of labor
- c) A prolapsed non pulsating umbilical cord
- d) A laboring woman with heart disease during the second stage of her labor
- e) After coming head in breech delivery

Which of the following is NOT an indication for episiotomy?

- a) When a primigravida is in labor
- b) Preterm delivery
- c) Forceps assisted vaginal delivery
- d) Breech delivery
- e) Vaginal delivery after complete perineal tear repair

SUPPORT 41

This method is used to assess the following:

- a) Pelvic capacity
- b) Cephalopelvic disproportion
- c) Fetal presentation
- d) The station of fetal head

You are caring for a 33-year-old woman at 12 weeks gestation. She smokes and drinks alcohol occasionally. Which of the following statements is true?

- a) Limit alcohol intake to one or two glasses of wine is advisable
- b) Children with fetal alcohol syndrome have developmental, behavioral, and learning difficulties
- c) Alcohol consumption in the third trimester is not directly associated with fetal alcohol syndrome
- d) Smoking must be discontinued at least 3 months before pregnancy to avoid its hazards

Regarding nutritional counseling in pregnancy, which of the following is true regarding anemia?

- a) Anemia is diagnosed in the first trimester if hemoglobin is less than 11 g/dL
- b) In the third trimester, hemoglobin <12 g/dL indicates anemia
- c) Iron supplements are routinely prescribed from the 1st trimester in combination with folic acid
- d) Intravenous iron is better recommended than oral iron for most pregnant women with iron deficiency
- e) Anemia during pregnancy is considered severe when Hb <9 g/dL and needs urgent referral

SUPPORT 41

According to the Caldwell-Moloy classification of types of pelvis, android pelvis represents:

- a) 25%
- b) 15%
- c) 5%

A 32-year-old primigravida at 40 weeks gestation with unremarkable pregnancy asks about EDC accuracy. What is the margin of error for a 12-week ultrasound?

- a) 3 days
- b) 7 days
- c) 14 days
- d) 21 days

According to the picture above, this maneuver is called:

- a) Muller-Kerr's method
- b) Sims method
- c) Pinard method
- d) None of the above

This maneuver is done while the patient is in what position?

- a) Semi sitting position
- b) Sims position
- c) Dorsal position

A 22-year-old primigravida develops insomnia, fatigue, and crying spells on postpartum day 4 without suicidal ideation. What is the most likely diagnosis?

- a) Postpartum blues
- b) Postpartum depression (PPD)
- c) Postpartum psychosis
- d) Alcohol withdrawal

SUPPORT 41

All of the following are risk factors for postpartum depression EXCEPT:

- a) Prior psychiatric disease of any kind
- b) The postpartum blues
- c) Cesarean section
- d) Exclusive bottle feeding
- e) Infant with special needs

Which of the following statements is NOT true regarding the use of any estrogen-containing hormonal contraceptive method for this patient?

- a) Hormonal is associated with a significantly decreased risk of PID
- b) Estrogen may decrease her risk of endometrial cancer
- c) History of chlamydia is not a contraindication for hormonal contraception
- d) Estrogen is contraindicated because of her obesity
- e) Estrogen may improve her acne

Which would be the most appropriate contraceptive method for this patient at this time?

- a) Bilateral tubal ligation (BTL) or vasectomy
- b) Transdermal contraceptive patch or vaginal contraceptive ring
- c) COC pills
- d) A levonorgestrel IUD or Depo-Provera

SUPPORT 41

- Female patient 33 yrs. old coming to the delivery room, she is G5 P4+0 pregnant 38 weeks. All her previous delivery were at home with the mid wife help. She neglect some of her ANC visit. IN the last visit the HBA1C was 6.8 and her doctor said that she is now diabetics and the fetal weight is above the 95 centile according to the ultrasound measurement and it is better to deliver at hospital due to her condition but she prefer to deliver at home as her previous deliveries. During her examination patient look exhausted, elevated temperature and respiratory rate, cervical edema, with fetal distress.

1. According to the case above, it represents an example of abnormal labor called:

- a) Obstructed labor
- b) Precipitated labor
- c) Delayed labor
- d) Augmented labor

2. The best method for delivery of baby in this case is by:

- a) Trial of labor
- b) Vaginal delivery using ventouse
- c) Vaginal delivery using forceps
- d) Caesarean section

3. There is a characteristic sign during examination of this patient called:

- a) Cervical ring
- b) Bandl's ring
- c) Schroeder's ring

4. The best method for delivery of this patient is instrumental vaginal delivery:

- a) True
- b) False

SUPPORT 41

Which is the incorrect statement regarding instrumental vaginal delivery?

- a) More common with the usage of epidural anesthesia
- b) Vacuum extraction requires less pain relieving than forceps delivery
- c) Proper usage of oxytocin limits the need for instrumental delivery
- d) Increases the rate of perineal tears
- e) Maternal trauma is more frequent with vacuum extractor than with obstetric forceps

Which is a contraindication for forceps delivery?

- a) Engaged presenting part
- b) Direct mentoposterior position
- c) Fully dilated cervix
- d) Ruptured fetal membranes
- e) Empty urinary bladder

Which is the correct statement regarding obstetric forceps?

- a) Used only in vertex presentation
- b) Should be used before engagement of the fetal head
- c) Contraindicated with preterm birth
- d) Can be used during cesarean section
- e) May be used in brow malpresentation

Which is the incorrect statement regarding traction with the forceps?

- a) Traction should be intermittent
- b) Episiotomy is needed
- c) Delivery of fetal head should be slow
- d) Apply traction only during uterine contraction
- e) The fetal head should be extended

SUPPORT 41

Which of the following is not a prerequisite for a forceps delivery?

- a) Engagement of fetal head
- b) The occipito anterior position
- c) A completely dilated cervix
- d) Ruptured membranes
- e) An empty bladder

Which is a contraindication of vacuum extractor?

- a) Fetal distress in the second stage
- b) Borderline cephalopelvic disproportion
- c) Shortening of the second stage in hypertensive mothers
- d) Suspected fetal coagulopathy
- e) Occipito-posterior position

Which is the incorrect statement regarding preoperative preparation for cesarean section?

- a) The bladder must be full to identify the lower uterine segment
- b) Intravenous fluids to keep normal hydration
- c) Skin area is prepared with antiseptic solution
- d) Blood transfusion is prepared for high risk pregnancies
- e) Confirm empty stomach

Which is the incorrect statement regarding anesthesia for cesarean section?

- a) Local anesthesia could be used in conditions
- b) Maternal position during the procedure is very important
- c) Anesthetic complication is significant cause of maternal mortality
- d) Postoperative pulmonary aspiration is a significant cause of maternal mortality
- e) Epidural anesthesia is associated with high risk of postoperative aspiration

SUPPORT 41

Which is the incorrect statement regarding postoperative care after uncomplicated cesarean section?

- a) Maintain intravenous fluids to avoid dehydration
- b) Monitor urine output
- c) Abdominal palpation to confirm uterine contraction
- d) Early ambulation is encouraged
- e) Breathing exercises are to be started only after the puerperium

Which is the incorrect statement regarding cesarean section?

- a) Safer than vaginal delivery for breech delivery
- b) Associated infection is markedly reduced by the use of prophylactic antibiotics
- c) Elective Cs under regional anesthesia is safe for the mother as normal delivery
- d) Commonly performed for prolonged delivery
- e) The lower segment Cs is safer than upper segment Cs

Which is NOT an indication of cesarean section?

- a) Failed forceps delivery
- b) Previous two cesarean sections
- c) Fetal distress
- d) Old cervical tear
- e) Cord prolapse

Which is NOT an indication of cesarean delivery?

- a) Failure of trial of forceps
- b) Previous classic cesarean section
- c) Persistent late deceleration of the fetal heart rate in the first stage of labour
- d) Vaginal prolapse
- e) Prolapsed pulsating cord in the first stage of labour

SUPPORT 41

Which is NOT a cause for the current high rate of cesarean section?

- a) Increased prevalence of contracted pelvis
- b) Decreased chances for vaginal delivery of breech presentation
- c) Decreased chances for vaginal delivery in case of fetal distress
- d) Increased prevalence of women with previous cesarean delivery
- e) More safe anesthetic techniques

Why is the uterine scar following lower uterine segment Cs stronger than that following upper segment Cs?

- a) Proper surgical coaptation of the edges at repair of the upper segment incision
- b) Hematoma formation is less likely with the lower segment Cs
- c) The upper segment is passive during puerperium
- d) The myometrium of the upper stronger than that of the lower segment
- e) The upper segment incision is easier to repair

Which is the advantage to the upper segment cesarean section over the lower segment cesarean section?

- a) Less possibility of primary bleeding
- b) Less possibility of paralytic ileus
- c) Safer in the presence of pelvic adhesions
- d) Lower incidence of subsequent adhesions
- e) Lower possibility of infection

Which is NOT one of the prerequisites for vaginal birth after CS?

- a) Current singleton pregnancy
- b) Previous Cs was LSCS and not USCS
- c) Current vertex presentation
- d) Previous only one CS
- e) Fetal weight not less than 4 kg in the current pregnancy

SUPPORT 41

Which is NOT an indication of Classical CS?

- a) Pelvic adhesions
- b) Anterior placenta previa
- c) Previous lower segment CS
- d) Neglected shoulder presentation
- e) Previous successful repair of vesico-vaginal fistula

Which is NOT an indication of cesarean hysterectomy?

- a) Uterine atony not responding to medical treatment
- b) Placenta accreta
- c) Uterine rupture that cannot be repaired
- d) Multiple fibroid uterus
- e) Bicornuate uterus

Which is the incorrect statement regarding obstetric forceps?

- a) It has traction action
- b) Cephalopelvic application is the safest application
- c) Low forceps is applied for station +2 or more
- d) Mid-forceps is applied for the unengaged head
- e) Can be applied for a malrotated head

Which is NOT a cause for failed forceps delivery?

- a) Contracted pelvis
- b) Uterine inertia
- c) Short umbilical cord
- d) Cephalopelvic disproportion
- e) Contraction ring

SUPPORT 41

- True & False

1. Mediolateral and median types of episiotomy have the same level of repair skills requirements.
2. In Egypt the incidence of CS is 40%.
3. Breech malpresentation is one of the ABSOLUTE indications of CS.
4. Selective CS is done during labor.
5. Forceps may be applied for some delivery cases of face presentation.
6. Cephalohematoma is more common with ventouse than with forceps delivery.
7. Piper forceps has a longer pelvic curve than other forceps types.
8. Upper segment CS can provide a faster access than lower segment CS for obstetric cases with fetal distress.
9. There are two transverse skin incisions which can be used by the obstetrician for lower segment CS.
10. The most common first fetal part seen during upper segment CS is fetal head.
11. Ventouse can be used for operative vaginal delivery of fetus with gestational age of completed 37 weeks and vertex presentation.
12. The two recti muscles are incised transversely during lower segment CS.

SUPPORT 41

13. Kielland forceps is used for outlet forceps delivery.

14. Pelvic curve of a single forceps blade is the concave part which comes in contact with fetal head.

15. The flexion point for ventouse cup application is at the occipital area.

16. The incidence of ruptured uterus in next pregnancy after upper segment CS is 0.4%.

17. CS is not contraindicated in a pregnant woman at 35 weeks' gestation with intrauterine fetal death and placenta previa centralis. She has previous two vaginal deliveries.

• Short Answer

1. List four major causes of Cesarean section.

2. List two contraindications of CS.

3. List three types of anesthesia for CS.

4. List two types of CS either according to uterine incision or timing of the procedure.

5. List three uterine incisions which can be done during CS.

6. Name the instrument referred to by blue arrow which was used during shown CS image. Why is it used?

7. What is the range of uterine rupture in deliveries following upper segment CS?

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8. What is the range of uterine rupture in deliveries following lower segment CS?
9. At what gestational age is elective CS recommended to be done?
10. List five prerequisites before forceps application.
11. List four complications of forceps application.

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- Case 1

30-year-old G2P1+0 at 14 weeks presented to the delivery suite complaining of mid spotting and lower abdominal cramps. On examination, her cervix was closed.

1. What is your differential diagnosis?
2. What is the most probable diagnosis?
3. What is the management of this condition?

- Case 2

25-year-old female at 8 weeks' pregnancy comes to ER complaining of fever, rigors, and foul-smelling discharge. On history taking, she tried to induce self-abortion outside the hospital. Temperature is 40°C, pulse 120 bpm, and ultrasound shows heterogeneous intrauterine content.

1. What is the most probable diagnosis?
2. Mention three complications?
3. What is the management of this condition?

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- Case 3

23-year-old female with amenorrhea for 7 weeks complains of spotting and lower abdominal pain. Vitals: pulse 120 bpm, BP 80/50 mmHg. Ultrasound shows moderate intraperitoneal collection and empty uterus.

1. What is your differential diagnosis?
2. What is the most probable diagnosis?
3. What is the management in the ER?

- Case 4

25-year-old woman G2P0 presents to the emergency room with unilateral pelvic and left lower abdominal pain. She admits to recent vaginal bleeding. She is afebrile and hemodynamically stable. Qualitative serum beta-hCG is positive.

1. What is the most likely diagnosis?
2. What further investigation is needed?
3. What is your management?

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- Case 5

A 26-year-old woman at 7 weeks of pregnancy presents to casualty complaining of bright red vaginal bleeding and suprapubic tenderness. On examination, there is blood in the vagina and the cervix is closed.

1. What is your differential diagnosis?
2. What is the most probable diagnosis?
3. What is the next step of management?

- Case 6

G3P2 pregnant at 7 weeks + 3 days presented to ER in agony. On examination, generalized abdominal tenderness was noted. Beta-hCG titer was 1000.

1. What is your provisional diagnosis?
2. How would you confirm the diagnosis?
3. What is the main line to treat this case?

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- Case 7

A 34-year-old woman, 7 weeks pregnant, presents to casualty. She has a history of pelvic inflammatory disease. She is complaining of left-sided abdominal pain and brown discharge. On examination, she is tender in the left iliac fossa and the cervix is closed.

1. What is the most likely diagnosis?

2. What further investigation is needed?

3. What is your management?

- Case 8

A 36-year-old woman presents with a history of one preterm birth of a healthy infant at 33 weeks followed by three consecutive miscarriages at 8–10 weeks gestation over the past 4 years. All miscarriages required curettage.

1. What is the most likely diagnosis?

2. What investigations will you offer this patient?

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- Case 9

A 26-year-old woman, 10 weeks pregnant, presents to casualty complaining of severe vaginal bleeding and suprapubic pain. On examination, blood is noted in the vagina. Pulse: 110 bpm, BP: 160/110 mmHg.

1. What is the most likely diagnosis?
2. What further investigations are needed?
3. What is your management?

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Regarding indications of cesarean hysterectomy, which of the following statements is NOT true?

- a) The mere presence of couvelaire uterus.
- b) Placenta accreta encountered during cesarean section
- c) Uncontrolled postpartum hemorrhage
- d) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- e) Operable carcinoma of the cervix during pregnancy

Which of the following is NOT an absolute indication for cesarean delivery?

- a) When the true conjugate diameter of maternal pelvis is less than 6 cm
- b) Major degree of placenta previa associated with an intrauterine fetal death
- c) Fetal macrosomia
- d) Cervical fibroid measuring 20 cm
- e) Conjoined twins

Which of the following is NOT an early complication of cesarean delivery?

- a) Total intraoperative blood loss of 580 ml
- b) Allergic reaction to anesthetic agents
- c) Amniotic fluid embolism
- d) Uterine artery injury
- e) Bladder injury



Mrs. G has undergone a shown procedure during her delivery. Which is correct regarding this procedure?

- a) Vulvar hematoma is a possible sequel
- b) When the second stage is more than an hour, it is indicated
- c) Permanent “non-absorbable” suture type is required
- d) Spinal anesthesia is required
- e) It is not recommended for preterm delivery

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To reduce post cesarean infectious morbidity, within how many minutes should antibiotic prophylaxis be given prior to cesarean delivery?

- a) 90 minutes
- b) 30 minutes
- c) 10 minutes
- d) 60 minutes

Dr. K is performing an elective lower segment cesarean section for his patient because of complete breech malpresentation. Considering the attached image, what anatomical layer is Dr. K incising by his scalpel?

- a) Subcutaneous tissue layer
- b) Rectus sheath layer
- c) Parietal layer of peritoneum
- d) Muscle layer
- e) Uterine wall



A 22-year-old laboring woman is undergoing the following shown procedure. What is correct about this procedure?



- a) It is indicated when second stage of labor exceeds 20 minutes
- b) It depends on positive pressure suction of the cup
- c) Fetal cephalohematoma is more common with it than forceps delivery
- d) It can be applied to breech malpresentation
- e) The cup is applied at parietal eminence of fetal vertex

Which of the following is NOT an indication for episiotomy?

- a) Forceps assisted vaginal delivery
- b) Vaginal delivery after complete perineal tear repair
- c) Breech delivery
- d) When a primigravida is in labor
- e) Preterm delivery

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Regarding a shown instrument in this image, which feature is true?



- a) Its material may be metal, plastic and rubber
- b) The shank is the part which determines the type of blades locking into each other
- c) Its pelvic curve is the part applied to fetal head
- d) It has a perineal curve
- e) The shown instrument is suitable for an outlet delivery procedure

Which of the following is NOT an indication for forceps assisted vaginal delivery?

- a) A laboring woman with heart disease during the second stage of her labor
- b) A second stage more than 190 minutes
- c) After coming head in breech delivery
- d) A fetal heart rate of 60 beats per minute during the second stage of labor
- e) A prolapsed non-pulsating umbilical cord

وَأَخِرُ دَعْوَاهُمْ أَنِ الْحَمْدُ لِلَّهِ رَبِّ الْعَالَمِينَ